

Colorectal Cancer Strategy

National Statistics

HRSA Data 2024

42.71% CRC Screening Rate for 3.6 M Patients Screened

(Last 5 years has been in the 40.42%-42.82% range for 2.4 to 3.6M patient range)

<https://data.hrsa.gov/topics/healthcenters/uds/overview/national>

GPRA 2024

23.6% (National target of 23.7%)

<https://www.ihs.gov/quality/government-performance-and-results-act-gpra/gpra-report-summary-2024/>

2023 was 23.3%

National Data

59% of adults are up-to-date on screenings

National goal is 80%

"In the U.S., screening is lowest among ages 45-49 (20%) and ages 50-54 (50%); Asian Americans (50%); American Indian or Native Alaskan (52%); Hispanic (52%); individuals with less than a high school education (48%), individuals with a lower income (<100% Federal Poverty Level) (47%); the uninsured (21%); and recent (<10 years) immigrants (29%)."

Colorectal cancer is swiftly shifting to more advanced disease and younger individuals.

- The proportion of individuals in the United States diagnosed with advanced-stage colorectal cancer increased from 52% in the mid-2000s to 60% in 2019.
- Early age onset colorectal cancer is on the rise. Diagnoses of people under 55 years of age doubled from 11% (1 in 10) in 1995 to 20% (1 in 5) in 2019.
- Colorectal cancer is now the leading cause of cancer death in men and the second in women under 50 years old.

<https://nccrt.org/our-impact/80-in-every-community/>

"In the United States, colorectal cancer (CRC) is the **third most diagnosed cancer** and the **third leading cause of cancer death** in both men and women." <https://www.cancerresearch.org/blog/colorectal-cancer-awareness-month>

"One in five people diagnosed with CRC are now under the age of 55."

About (From the Colorectal Cancer Alliance) <https://colorectalcaner.org/basics/health-equity>

"Data shows us the impact of disparities:

- Black Americans are 15% more likely to get colorectal cancer and 35% more likely to die from it
- Hispanic Americans are screened at lower rates, with only slightly more than 50% of those eligible checked for colorectal cancer
- The racial disparity in incidence for both sexes combined widened for AI/AN individuals from 21% to 41%
- Pervasive stigma, discomfort with screening methods, lack of institutional trustworthiness, and bias/racism in the healthcare system
- Patients with a lower economic status of any race are at substantially higher risk for undertreatment and treatment delay"

<https://pmc.ncbi.nlm.nih.gov/articles/PMC9069392/>

Resources for Patients

Know Your Risk for all Cancers: <https://acscancerrisk360.cancer.org/>

Screening Quiz: <https://colorectalcaner.org/screening-quiz>

Patient Navigator (Spanish Option): "Have questions now? Call our free Helpline at (877) 422-2030 to speak with certified patient navigators. " <https://colorectalcaner.org/resources-support/resources/live-navigator-helpline>

Support Group:

- https://www.cancercare.org/support_groups/45-colorectal_cancer_patient_support_group
- <https://bluehq.org/>
- <https://colorectalcaner.org/resources-support/community-support/buddy-program>

Financial Assistance for expenses (\$200) or screening: <https://colorectalcaner.org/resources-support/resources/financial-assistance>

Information on insurance coverage: <https://www.cancer.org/cancer/types/colon-rectal-cancer/detection-diagnosis-staging/screening-coverage-laws.html>

Other Educational Links

<https://www.cancerresearch.org/blog/colorectal-cancer-awareness-month>

<https://www.globalcca.org/learn/colorectal-cancer> (Great Visualizations)

<https://colorectalcancer.org/sites/default/files/media/documents/2025%20Screen%20Smart%20Discussion%20Slides.pdf>

<https://colorectalcancer.org/get-involved/advocate/become-advocate/screen-smart>

<https://www.cancer.org/cancer/types/colon-rectal-cancer/detection-diagnosis-staging.html>

Data visualizations for national data: <https://seer.cancer.gov/statfacts/html/colorect.html>

Interactive map for hot spots: <https://statecancerprofiles.cancer.gov/map/map.withimage.php?00&county&001&020&00&0&02&0&1&5&0#results>

Colon Cancer Coalition: <https://www.youtube.com/@GYRIGvideo/videos>

CCA Privacy Policy: <https://tandcs.us/ccalliance/>

INTERNAL DATA RESEARCH: https://docs.google.com/document/d/12H-AkdaEqnpSq_xXjA9ERu0POumHuBYfsHlm-8swF38/edit?tab=t.0

Prevention

Risk Factors we can change <https://www.cancer.org/cancer/types/colon-rectal-cancer/causes-risks-prevention/risk-factors.html>

- Body weight
- Type 2 Diabetes
- Diet
 - Higher consumption of processed and red meats, and cooking meats at very high temperatures (frying, broiling, grilling)
 - Low blood level of vitamin D
 - Lower intake of fruits, vegetables, and whole grains.
- Smoking
- Alcohol - moderate to heavy use
- Exercise - sedentary lifestyles

Factors you cannot change

- **Age** - More common after age 50, although it is rising among younger people
- **Race/Ethnicity** - American Indian and Alaska Native people have the highest rates of colorectal cancer in the United States, followed by African American men and women. Jews of Eastern European descent (Ashkenazi Jews) have one of the highest colorectal cancer risks of any ethnic group in the world.
- **Sex at birth** - Men who have colorectal cancer are more likely to die from it than women. The reasons are not fully clear. Women who have colorectal cancer are more likely to have right-sided colon cancer, particularly if they are no longer menstruating (postmenopausal).
- **Gallbladder removal** - People who have had their gallbladder removed (cholecystectomy) have been found to have a mildly higher risk for right-sided colon cancer. It's not fully understood why this is. Research is ongoing.
- **History of Polyps of Colorectal Cancer** - This is especially true if the polyps are large, if there are many of them, or if any of them show dysplasia. If you've had colorectal cancer, even though it was completely removed, you are more likely to develop new cancers in other parts of the colon and rectum. The chances of this happening are greater if you had your first colorectal cancer when you were younger.
- **Inflammatory Bowel Disease** - If you have inflammatory bowel disease (IBD), including either ulcerative colitis or Crohn's disease, your risk of colorectal cancer is increased. If you have IBD, you may need to start getting screened for colorectal cancer when you are younger and be screened more often. Inflammatory bowel disease is different from irritable bowel syndrome (IBS), which does not appear to increase your risk for colorectal cancer.
- **History of Radiation to the abdomen or pelvis** - If you survived cancer in the past and as part of your treatment, received radiation to the area where your colon is (abdomen and pelvis area), your risk of colorectal cancer is increased. If you have received radiation to the abdomen or pelvis, especially as a child, you may need to start getting screened for colorectal cancer when you are younger and be screened more often.

Several studies suggest that men who had radiation therapy to treat prostate cancer might have a higher risk of rectal cancer because the rectum receives some radiation during treatment. Most of these studies are based on men treated in the 1980s and 1990s, when radiation treatments were less precise than they are today. The effect of more modern radiation methods on rectal cancer risk is not clear, but research continues to be done in this area.

- **Family History** - Most colorectal cancers are found in people without a family history of colorectal cancer. Still, as many as 1 in 3 people who develop colorectal cancer have other family members who have had it. People with a history of colorectal cancer in a first-degree relative (parent, sibling, or child) are at increased risk. The risk is even higher if that relative was diagnosed with cancer when they were younger than age 50, or if more than one first-degree relative is affected. Having family members who have had adenomatous polyps is also linked to

a higher risk of colon cancer. (Adenomatous polyps are the kind of polyps that can become cancer.)

- **Inherited Syndrome** - The most common inherited syndromes linked with colorectal cancers are Lynch syndrome (hereditary non-polyposis colorectal cancer, or HNPCC) and familial adenomatous polyposis (FAP), but other rarer syndromes can increase colorectal cancer risk, too.

Other

- **Environmental Quality** - "This study evaluated a population-based ecological approach and showed that YOCRC was associated with lower environmental quality, including built and water domains. EQI domains were also associated with different racial groups among YOCRC. Instead of studying single environmental exposures, the impact of the cumulative environmental index should be recognized and studied further to understand the mechanism of cancer development." <https://meetings.asco.org/abstracts-presentations/208438>

Screening

"Screening for CRC is highly cost-effective to reduce the prevalence of CRC in the general population [2,24]. Depending on the type of test utilized to screen, screening may find cancers and precursor adenomas, and locate CRCs at earlier stages compared to unscreened patients [2,3,24]. Colonoscopy and FIT are considered first-tiered screening tests, with fecal DNA tests, computed tomography colonoscopy, and flexible sigmoidoscopy considered second-tiered tests [3,24]. Colonoscopy is the ultimate test to be completed irrespective of the initial screening positive test due to its ability to completely visualize the colon, and is diagnostic and therapeutic. Because CRCs generally begin as adenoma precursors, there is evidence that removal of adenomas (via polypectomy) that may result from screening reduces the incidence and mortality from CRC, and the protective effect from CRC mortality is durable [15,94,95]."

"Black Americans within the PLCO cancer screening trial were less likely than White Americans to have a follow-up colonoscopy within 1 year of an abnormal screening test [96]."

"Navigation programs have been shown to substantially improve colonoscopy completion rates [21,102-104]. The Delaware Cancer Consortium performed 10,000 patient navigations for colonoscopic CRC screening among Black and White Americans over a 9-year period [21]. There was a dramatic change in stage at diagnosis for Black Americans during the 9-year period, with distant CRC disease dropping from 23% to 7%, regional disease dropping from 56% to 33%, and local disease rising from 15% to 50% where CRC is curable [21]. Furthermore, over the 9-year study, the overall number of patients navigated rose for both Black Americans (47.8% to 73.5%) and White Americans (58.0% to 74.7%) and the CRC incidence dropped for Black Americans (from 68/100,000 to 48/100,000) and White Americans (from 60/100,000 to 48/100,000), totally eliminating the CRC incidence disparity [21]. In a similar vein, the CRC mortality, generally a lagging measure to incidence, dropped for Black Americans (from 31.27/100,000 to 18.35/100,000) and for White Americans (from 19.45/100,000 to 16.94/100,000), nearly eliminating the mortality disparity [21]. This remarkable study shows the power of navigation to raise CRC screening for all subpopulations, and essentially proves that increasing the rate of CRC screening for the whole population can eliminate the disparities in CRC incidence and mortality."

<https://pmc.ncbi.nlm.nih.gov/articles/PMC9069392/>

- Early signs to look for before 45
 - The American Cancer Society reports that the incidence of colorectal cancer in adults aged 20-39 has been rising at about 2% annually since the mid-1990s. They also identified that colorectal cancers have become the leading causes of cancer death in men and second in women behind breast cancer in people under 50.
 - <https://www.tampabay.com/sponsored/2025/02/17/colorectal-cancer-spike-among-millennials/>
 - **Race and Ethnicity:** In the U.S., younger Black adults have consistently higher incidence rates over time.[16] However, some studies show significant increases in incidence among young non-Hispanic Whites and Hispanic men.[17] Younger adults diagnosed with colorectal cancer are more likely to be Black, Asian/Pacific Islander, or Hispanic compared to older patients.[14]
 - **Rectal bleeding:** This had the strongest association with an early-onset diagnosis.[3] It can appear as bright red blood in the stool or make the stool look darker.[8]
 - **Abdominal pain:** This was the most common sign reported by young people with colorectal cancer.[3] It can manifest as cramps, gas, or general discomfort.[5][9]
 - **Diarrhea:** A persistent change in bowel habits, including ongoing diarrhea, is a significant indicator.[3][4]
 - **Iron-deficiency anemia:** This can result from the cancer bleeding into the digestive tract and may be the first sign of the disease, often detected through a blood test.[3][8]
 - Other symptoms to be aware of include a change in the consistency or shape of stool (such as narrow, ribbon-like stools), a feeling that the bowel doesn't empty completely, unexplained weight loss, and persistent fatigue.[5][10] It's crucial to note that these symptoms can also be caused by other conditions like hemorrhoids or irritable bowel syndrome.[8] However, given the rising incidence in younger people, it is vital not to ignore them.[10]
- Timely screening for all at 45
- Patient Navigators
 - <https://ascopubs.org/doi/full/10.1200/JCO.2012.47.8412>
 - <https://link.springer.com/article/10.1007/s11606-012-2044-2>

Overcome physician bias for recommendation by messaging everyone via CareMessage

- Address patient fear and mistrust of screenings
- Address cost of the various options along with pros/cons

Ideal combination is to pair messaging with navigators (Professional or Community-based Peer Navigators)

Automate follow-up for a positive non-invasive test

Address cost of a colonoscopy when billed as screening vs. diagnostic

<https://ecqi.healthit.gov/sites/default/files/ecqm/measure/CMS130v13-eCQM-Flow.pdf>

- <https://academic.oup.com/her/article-abstract/28/5/803/616979>
- Cost of screenings - <https://www.cancer.org/cancer/types/colon-rectal-cancer/detection-diagnosis-staging/screening-coverage-laws.html>
 - Covered by all health insurance companies under the ACA
 - Must be recommended by USPSTF
 - Colonoscopies may be billed as screenings vs. diagnostic (ex. polyp is removed)
- Follow-up and interpreting results
- Options
 - **Genetic Counseling** - If you have a family history.
 - **Biomarker Testing** - <https://www.globalcca.org/programs/know-your-biomarker>
 - **Colonoscopy** - A visual examination of the entire colon and rectum. It is highly effective at detecting both cancer and precancerous polyps.
 - Invasive, requires a day off work, and has associated costs. Transportation and childcare can be significant barriers for low-income individuals.
 - "Multifaceted points of communication and execution: (a) provides general education about the procedure, including its importance in reducing cancer risk, (b) ensure prep is picked up and/or delivered, (c) instructions and coaching on prep utilization and completion, (d) arrange transportation to and from colonoscopy site, (e) arrange observer post-procedure with follow-up contact within hours post procedure" <https://pmc.ncbi.nlm.nih.gov/articles/PMC9069392/>
 - **Fecal occult blood test (FOBT) or Fecal Immunochemical Test (FIT)** - An at-home stool test that detects hidden blood.
 - Non-invasive and can be done at home, increasing convenience. Follow-up colonoscopy is required for a positive result, which can be a barrier.
 - **Stool DNA Test (Cologuard)** - An at-home test that detects altered DNA from cancer or polyps in the stool.
 - More sensitive than FIT but also has a higher rate of false positives, leading to more follow-up colonoscopies.
 - **Blood Test (Shield by Guardant)** - <https://www.fda.gov/medical-devices/recently-approved-devices/shield-p230009>
 - Not for those less than 45 or with a high risk of colorectal cancer

White (64.5%) and Asian patients (68.1%) had the highest rates of colonoscopy utilization, whereas Latino (33.2%), African American (49.5%), and Other patients (50.1%) had significantly lower rates ($p < 0.001$). <https://pmc.ncbi.nlm.nih.gov/articles/PMC11679185/>

- Use of SMS
 - With AI/AN men - "The high observed mortality for screen detectable cancers in American Indians and Alaska Natives may be partly due to low uptake of cancer screenings." <https://www.mdpi.com/1660-4601/19/11/6546> High rates of both colorectal and prostate cancer, with very low screening rates, particularly in the Southwest region.
 - "Overall CRC screening rate was 44.3% for American Indian/Alaska Native men across the nation, but CRC screening rate was 36.6% among American Indian/Alaskan Native men in the Southwest. Reasons for low screening rates may include stigma and fear related to diagnosis, distrust of medical professionals, lack of knowledge or awareness, masculinity, lack of transportation, and geographic distance to screening facilities [23,24,25,26]. American Indians also have multiple challenges to healthcare (e.g., lack of healthcare coverage, language and culture, poverty, and discrimination) resulting in low healthcare utilization [21,27,28,29,30]. American Indians are less likely to utilize preventive care or attend healthcare provider appointments /follow-ups [29]. However, a few studies investigated factors contributing to low cancer screening rates specifically for American Indian men [22,25], and factors influencing cancer screening behavior may vary in different American Indian tribes."
 - "The use of Native Patient Navigators and community education have been demonstrated to increase cancer care utilization including screening and to shorten the time between an abnormal finding and diagnosis among American Indians [31,32,33]. Use of mobile health (mHealth) technology, such as text messaging, has also been successfully used in interventions for health promotion [34], including efforts to increase cancer screening in American Indians and Alaska Natives [35]."

Treatment

- <https://www.globalcca.org/learn/colorectal-cancer>
- Navigation - Cost of treatment, Treatment Options, Health Insurance, Transportation, Financial support for lost wages
- Financial Toxicity - <https://coloncancerfoundation.org/the-unintended-burden-of-a-cancer-diagnosis-financial-toxicity/>
 - "Nearly 3 out of 4 mCRC patients experienced (major financial hardship)MFH despite access to health insurance. These findings underscore the need for clinic and policy solutions that protect cancer patients from financial harm." <https://academic.oup.com/jnci/article/114/3/372/6492636?login=false>
 - <https://colorectalcancer.org/resources-support/resources/financial-assistance/about-financial-toxicity>

- Treatment options
 - "Genetics may be one reason why CRC in Black patients can be difficult to treat. Additional research from Memorial Sloan Kettering found that colon cancer patients of African ancestry may have tumors that don't respond well to [immunotherapy](#) and [targeted cancer therapy](#). The researchers found that these patients' tumors were less likely to have the molecular profiles needed for these treatments to work. But more research is needed. For now, researchers have very few clues as to why, when, and how these molecular and biological differences of CRC exist among various racial/ethnic and ancestral backgrounds, he said." <https://www.mdedge.com/oncologypractice/article/266396/gastroenterology/breaking-barriers-colorectal-cancer-care-black>
 - <https://www.cancer.gov/about-cancer/treatment/types/targeted-therapies>
 - <https://my.clevelandclinic.org/health/treatments/11582-immunotherapy>
- Timeliness of care
- Clinical Trials
 - https://www.fredhutch.org/en/research/clinical-trials.html?q=%22colon%20cancer%22%20%22colorectal%20cancer%22&size=n_10_n&filters%5B0%5D%5Bfield%5D=search_result_type&filters%5B0%5D%5Btype%5D=any&filters%5B0%5D%5Bvalues%5D%5B0%5D=Clinical%20Trials
- Stage III (90%) or high-risk Stage II (10%) Resection (removal) and adjuvant chemotherapy
 - Exercise vs. Education (Study in Canada) - "A 3-year structured exercise program initiated soon after adjuvant chemotherapy for colon cancer resulted in significantly longer disease-free survival and findings consistent with longer overall survival." <https://www.nejm.org/doi/pdf/10.1056/NEJMoa2502760>
 - "Those in the exercise group received the same materials plus an exercise guidebook developed for colon cancer survivors¹⁶ and support from a certified physical activity consultant for 3 years." <https://www.nejm.org/doi/10.1056/NEJMo008039/full>
 - "Patients in the health-education group received general health-education materials promoting physical activity and healthy nutrition in addition to standard surveillance."

Recovery

- Up to 40% of patients have disease recurrence

Palliative Care

- Decision-making on End-of-life care and advance care planning <https://pmc.ncbi.nlm.nih.gov/articles/PMC11679185/>
- "When it came to hospice utilization, a meta-analysis found that hospice services were mostly utilized by White patients, followed by Hispanics, and least used by African and Asian Americans. This study found that Black persons underwent aggressive treatment at the end of life; however, this was often inconsistent with their preferences." ⁴²

Studies Evaluated (APA Style Citations):

Batai, K., Sanderson, P. R., Joshweseoma, L., Burhansstipanov, L., Russell, D., Joshweseoma, L., & Hsu, C.-H. (2022). Formative Assessment to Improve Cancer Screenings in American Indian Men: Native Patient Navigator and mHealth Texting. *International Journal of Environmental Research and Public Health*, 19(11), 6546. <https://doi.org/10.3390/ijerph19116546>

Clemma J. Muller PhD, Renee F. Robinson PharmD, MPH, Julia J. Smith MS, Meghan A. Jernigan MPH, Vanessa Hiratsuka PhD, Denise A. Dillard PhD, Dedra Buchwald MD. (2016). Text message reminders increased colorectal cancer screening in a randomized trial with Alaska Native and American Indian people. *American Cancer Society Journals*. <https://acsjournals.onlinelibrary.wiley.com/doi/full/10.1002/cncr.30499>

Underuse of colorectal cancer screening among men screened for prostate cancer <https://acsjournals.onlinelibrary.wiley.com/doi/full/10.1002/cncr.25229> Effectiveness of a short message service intervention to motivate people with positive results in preliminary colorectal cancer screening to undergo colonoscopy: A randomized controlled trial <https://acsjournals.onlinelibrary.wiley.com/doi/full/10.1002/cncr.32043>

Text message reminders increased colorectal cancer screening in a randomized trial with Alaska Native and American Indian people <https://acsjournals.onlinelibrary.wiley.com/doi/full/10.1002/cncr.30499>

Formative Assessment to Improve Cancer Screenings in American Indian Men: Native Patient Navigator and mHealth Texting <https://www.mdpi.com/1660-4601/19/11/6546#B35-ijerph-19-06546>

Efficacy of an mHealth Intervention (BRAVE) to Promote Mental Wellness for American Indian and Alaska Native Teenagers and Young Adults: Randomized Controlled Trial https://mental.jmir.org/2021/9/e26158?utm_medium=cpc&utm_source=TrendMD&utm_campaign=TrendMD_Internal

Text Messaging Intervention for Mental Wellness in American Indian and Alaska Native Teens and Young Adults (BRAVE Study): Analysis of User Engagement Patterns <https://formative.jmir.org/2022/2/e32138>